SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Paclitaxel + cetuximab every 4 weeks

Treatment of recurrent or metastatic squamous cell cancer of the head and neck. · NCCP 00696 · version 2 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Paclitaxel and cetuximab aim to control head and neck cancer that has returned or spread and may help symptoms. Your team explains expected benefit, uncertainty and options in light of your previous treatment.

Schedule
Paclitaxel is given on Days 1, 8 and 15, with cetuximab on Days 1 and 15 of each 28-day cycle. There is no scheduled treatment dose on Day 22. Both medicines continue while appropriate, until progression or unacceptable side effects; the protocol does not set a fixed total number of cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cetuximab
EGFR-TARGETED ANTIBODY

- ☐ Infusion reactions, including anaphylaxis
- ☐ Acne-like rash, dry or cracked skin and nail infections
- ☐ Low magnesium and other mineral changes
- ☐ Eye inflammation
- ☐ Diarrhoea and dehydration
- ☐ Rare serious lung inflammation

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Peripheral nerve damage that may persist
- ☐ Infusion or allergic reactions
- ☐ Hair loss
- ☐ Muscle and joint aches
- ☐ Mouth soreness, bowel changes and fatigue

Discuss the individual aim, expected benefit, alternatives and uncertainties; common, serious and lasting effects; infusion reactions and infection; fertility and pregnancy; treatment burden; support medicines and urgent contacts. NCCP identifies this paclitaxel–cetuximab combination and schedule as off-label use; the team must confirm the current licensing position for your actual prescription. Discuss the reasons, evidence, alternatives and uncertainties and document the local consent process.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00696 v2.

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